

Pediatric Midterm: New Questions from Pages 45-104 (Solved)

104 questions from pages 45-104 of "Pediatric Midterm Solved" that do not repeat anything on pages 1-44. Repeats inside pages 45-104 are merged into one question; the page tag lists every page where it appears. "Partial recall" means the source only remembered some options or only the answer. Wording is from the source, lightly cleaned of typos. Highlighted option = the answer highlighted in the source.

Neonatology

1. A 7-day-old develops small red macules and papules that progress to white-yellow pustules. No erythema is noted.

What is the diagnosis? p92 partial recall

A. Erythema toxicum neonatorum

B. Transient pustular melanosis

The source itself wrote "not sure" next to this answer. Compare with the next question: erythema toxicum has an erythematous flare and eosinophils.

2. A 2-day-old full-term neonate has multiple firm yellow-white 1-2 mm papules with a surrounding erythematous flare on the trunk. Wright stain of the lesions shows numerous eosinophils. He is active and feeding well. What is the LIKELY diagnosis?

p94

A. Herpes simplex infection

B. Pustular melanosis

C. Erythema toxicum

D. Folliculitis

3. A newborn has a unilateral red patch covering the upper eyelid and forehead. What is the lesion?

p63, 77, 87, 98 partial recall

A. Port-wine stain

B. Salmon patch (stork bite)

4. A 4-day-old neonate has jaundice. Bilirubin was measured and is in the phototherapy zone. What is the next step?

p76 partial recall

A. Oral hydration and phototherapy

B. Exchange transfusion

5. A 2-week-old formula-fed neonate weighed 3.6 kg at birth and now weighs 2.8 kg. He sleeps around 18 hours a day.

What is your concern? p72

A. Sleep hours

B. Calorie intake

C. Ingestion of solid food

D. Vaccination

6. A newborn develops apnea. What is the cause?

p90 partial recall

A. Immature central (respiratory) control

B. Wide nasal area

7. A mother asks about complications of prematurity. Which statement is correct? p63, 100 partial recall

A. The child will not complete high school

B. Spastic diplegia is a risk in preterms

8. An 18-day-old baby girl, born at 27 weeks with a birth weight of 850 g, is on full nasogastric feeds. She develops abdominal distention, vomiting and a smear of blood in the stool. She is lethargic with frequent apneas and her abdominal girth increased by 2 cm in 2 days. Labs: thrombocytopenia, Na 130 mmol/L. What would you find on the X-ray?

p82 partial recall

A. Pneumotosis intestinalis

Only the answer was recalled in the source.

9. A 3-week-old infant has microcephaly, periventricular calcification on skull X-ray, and blindness. What is the most likely cause? p77

A. Bilateral subdural hemorrhage

B. Cerebral agenesis

C. Primary microcephaly

D. Congenital infection (cytomegalovirus)

Infections

10. A 4-year-old has fever and increased work of breathing (tachypnea, retractions). Chest X-ray confirms lobar pneumonia. What is the most likely etiology? p45

A. Streptococcus pyogenes

B. Streptococcus pneumoniae

C. Mycoplasma pneumoniae

D. Chlamydia pneumoniae

11. A 5-day-old girl presents to the ED with tachycardia and fever. The antibiotic given should target which organism?

p57, 101 partial recall

A. Streptococcus pneumoniae

B. E. coli

p101 asks the same idea: most serious bacterial infection in a febrile neonate = E. coli septicemia.

12. A neonate has tachycardia, tachypnea and capillary refill > 4 seconds (shock). Cultures grow gram-positive cocci. What is the organism? p71

A. Group B streptococcus

B. Streptococcus pneumoniae

C. Group A streptococcus

D. Streptococcus viridans

13. A 1-month-old has fever, irritability and poor feeding. CSF analysis shows features of viral meningitis (details not recalled). How would you interpret this CSF? p46

A. Suggestive of partially treated meningitis

B. Viral meningitis

C. Bacterial meningitis

D. Normal CSF

14. A previously healthy 7-year-old has fever and headache for 3 days and abnormal movements for 1 day. He is lethargic with decreased consciousness. CSF: clear, mild WBC elevation (mainly lymphocytes), normal glucose, slightly elevated protein. What is the most likely diagnosis?

p54, 66, 98

- A. Autoimmune encephalitis
- B. Tuberculous meningitis
- C. Viral encephalitis
- D. Bacterial meningitis

15. A child has fever, headache and behavioral changes. CSF is clear with normal glucose, slightly raised protein and raised lymphocytes. Imaging shows temporal lobe involvement. What is the best next step in management?

p73, 89 partial recall

- A. Lumbar puncture
- B. Antibiotics
- C. IV acyclovir

16. An unvaccinated 4-year-old girl has fever and a generalized pruritic vesicular rash in varying stages. Her immunocompromised sister had a painful vesicular rash on one side of her trunk. Which is true about this illness?

p48, 88

- A. Immunocompromised patients usually get a mild breakthrough illness
- B. The causative organism is an RNA virus
- C. Feco-oral route is the known mode of transmission
- D. It can be complicated by necrotizing fasciitis due to GAS infection

17. A 2-year-old with femoral osteomyelitis is on IV antibiotics. After 72 hours the fever resolves and inflammatory markers improve. What is the next step?

p48

- A. Continue IV antibiotics for 6 weeks
- B. Stop antibiotics and monitor clinically
- C. Switch to oral antibiotics and continue for 3-4 weeks
- D. Bone biopsy to confirm the diagnosis

18. A 3-year-old comes to the ER with 2 days of fever and submandibular lymphadenopathy. What is a red flag in lymphadenitis?

p57

- A. 2 days of fever
- B. Local tenderness
- C. Recent cold symptoms
- D. Lymphadenopathy lasting more than 4 weeks

19. Fever, sore throat and (image in the exam) unilateral, tender, erythematous lymphadenopathy. What is the most likely organism?

p81, 83, 85

- A. Adenovirus
- B. Staphylococcus aureus
- C. EBV
- D. Cat scratch disease

The exam showed an image, not reproduced in the source.

20. A 10-year-old develops fever, exudative tonsillitis, prominent lymphadenopathy and lethargy. Amoxicillin is prescribed; the next day a maculopapular rash appears. Abdominal exam shows splenomegaly. What is the most likely cause of the rash?

p75

- A. EBV infection with amoxicillin reaction
- B. Immune complex
- C. Streptococcal pharyngitis
- D. Parvovirus (fifth disease)

21. A child has prolonged fever, sore throat, a sandpaper maculopapular rash and strawberry tongue (scarlet fever). What is the causative agent?

p87, 91 partial recall

- A. Group A streptococcus
- B. Staphylococcus aureus
- C. Coxsackie

22. A 2-year-old has a rash on both feet and a few lesions on both hands and the tongue. They are vesicular and tender. He is well and other family members are healthy. What is the most likely cause?

p82

- A. Coxsackie A (hand-foot-mouth disease)
- B. Flea bites
- C. Mumps
- D. Ebola virus

23. An unvaccinated 4-month-old has fever, vomiting and diarrhea without blood or mucus. What is the likely causative agent?

p88 partial recall

- A. Influenza A
- B. Salmonella
- C. Rotavirus

24. An 18-month-old presents after a brief generalized tonic-clonic seizure, now postictal, temperature 40 °C. During the lumbar puncture (normal), he passes a large watery stool with blood and mucus. What is the most likely diagnosis?

p97

- A. Salmonella
- B. Enterovirus
- C. Rotavirus
- D. Shigella

25. A 5-year-old girl has fever, mild ear pain and ear discharge for 2 days. She looks well. Temp 38.5 °C, HR 150. Left ear: bulging, dull tympanic membrane with otorrhea. What is the next best step?

p60, 70

- A. High-dose amoxicillin for 10 days
- B. Symptomatic therapy and reassess after 48-72 hours
- C. Consult ENT for tympanic surgery
- D. Blood culture from ear and other tests

Key says B in both copies. Be aware: the AAP otitis media guideline recommends antibiotics for AOM with otorrhea at any age, which would make A the answer. Ask your doctor which one they expect.

26. A 1-year-old has otitis media (unilateral bulging, erythematous tympanic membrane, ear pulling), fever, and looks well. What is the next step? p88, 91, 102 partial recall

- A. Discharge
- B. High-dose amoxicillin for 10 days

27. A patient has fever. History, examination and investigations are all negative. What type of fever is this? p82, 83 partial recall

- A. Fever of unknown origin
- B. Persistent fever
- C. Intermittent fever

Immunization

28. Parents are concerned about their child receiving MMR. Which is a valid contraindication? p46

- A. Older brother with autism
- B. Illness with low-grade fever
- C. Short course of low-dose corticosteroids
- D. Anaphylaxis to a previous similar dose

29. A patient has IgA deficiency. Which vaccine can be given? p57

- A. Pneumococcal vaccine
- B. Yellow fever vaccine
- C. Oral polio vaccine
- D. BCG

30. Which is true about the PCV vaccine? p72 partial recall

- A. It works against 13 serotypes
- B. It cannot be given after antibiotics

31. What is true about the pneumococcal conjugate vaccine? p83 partial recall

- A. It is conjugated to a protein carrier

Only the answer was recalled in the source.

32. What is true about the meningococcal conjugate vaccine? p84 partial recall

- A. It is given at birth
- B. It is given to small children

33. A 4-year-old has not seen a doctor for a long time. He received only his 2-month and 4-month vaccines. What is appropriate? p74 partial recall

- A. Give the vaccines for his current age and catch up the missed ones
- B. Give the vaccines for his age only and ignore the missed ones
- C. Repeat the 2- and 4-month vaccines because of the long gap

34. A child is due for varicella and MMR today. The mother does not want two vaccines in one day and asks to come back in 2 weeks for the second one. Best response? p75

- A. If live vaccines are not given together, they must be separated by at least 28 days
- B. Live vaccines may be given within 4 days of each other
- C. Live vaccines may be given within 2 weeks of each other
- D. Giving more than one live vaccine on the same day increases adverse reactions

35. A child receives varicella immunoglobulin. What type of immunity is this? p87, 90

- A. Passive artificial
- B. Active artificial
- C. Passive natural
- D. Active natural

36. A child is planning to take the MMR vaccine. What could affect his vaccine response? p95 partial recall

- A. Immunoglobulin given last month

Only the answer was recalled. See the note in the answer-conflicts section about the earlier IVIG/Kawasaki question.

37. A 3-month-old (possibly preterm) spent a couple of weeks in the ICU and was discharged. The parents are worried about severe lower respiratory infection. What do you advise? p80 partial recall

- A. Stick with the vaccination schedule
- B. Palivizumab injection once a month during winter
- C. Omalizumab injection once a month during winter

38. What vaccine is given to children at 6 months? p95 partial recall

- A. BCG

Key says BCG. BCG is normally given at birth, so this key looks wrong or the question was misremembered. Check your local schedule table.

Respiratory and asthma

39. A 10-year-old was recently discharged after an asthma exacerbation. What advice helps prevent future exacerbations? p45

- A. Limit physical activity
- B. Start oral antibiotics at the onset of any URTI
- C. Start SABA as needed for cough and shortness of breath
- D. Provide a written asthma action plan

40. A 3-year-old with recurrent wheezing was admitted at 9 months with bronchiolitis. Parents ask if he will develop asthma. According to the modified asthma predictive index, which increases the risk? p56

- A. Male gender
- B. Prematurity
- C. Siblings diagnosed with asthma
- D. Wheezing not related to colds

41. Which is a risk factor for poor asthma outcome?

p58 partial recall

A. Incorrect inhaler technique

Only the answer was recalled in the source.

42. You follow a 13-year-old with asthma. Which of the following is a reliever therapy? p69

A. Seretide (salmeterol + fluticasone)

B. Fluticasone

C. Zafirlukast

D. Symbicort (budesonide + formoterol)

43. A 5-year-old is diagnosed with asthma and a bronchodilator is prescribed. How should it be delivered? p86, 93

A. Dry powder inhaler

B. Metered dose inhaler with spacer

C. Nebulizer with face mask

44. An asthmatic patient has gone 4 weeks without needing his medications, no night awakening, and asthma does not affect his life. What do you do with his treatment? p84 partial recall

A. Step down according to the asthma action plan

Only the answer was recalled in the source.

45. A 14-year-old with asthma on a bronchodilator exercises normally and has no night waking. He had one exacerbation last year treated in the ER. What is the state of his asthma? p96

A. Well controlled

B. Partially controlled

C. Poorly controlled

46. Parents of a child with asthma refuse treatment. What would you tell them at counseling? p95 partial recall

A. Treatment has been shown to have adverse effects

B. Exacerbations can cause airway damage and remodeling

47. A child had sudden wheezing and cough while alone in a room. Chest X-ray shows unilateral hyperinflation. Asthma therapy did not help. What is the next step? p70 partial recall

A. Bronchoscopy

B. Oral steroid

C. Higher dose of inhaled corticosteroid

48. A child has recurrent chest infections, failure to thrive and weight loss despite a good appetite. His skin tastes salty. What is the most likely diagnosis? p77, 80 partial recall

A. Cystic fibrosis

Only the answer was recalled in the source.

Cardiology and rheumatology

49. A 4-month-old seen for an ear infection has a heart murmur. Which feature most suggests it needs further investigation? p75

A. Presence of a thrill

B. Murmur disappears on lying flat

C. Body weight of 6 kg

D. Systolic murmur

50. A child has dysmorphic features of Down syndrome (low-set ears, flat nasal bridge). Which heart defect is most likely? p82, 83, 85, 94 partial recall

A. Atrioventricular septal defect

B. Tetralogy of Fallot

C. ASD

51. A 10-year-old has fever, multiple joint pain and swelling, tachycardia, tachypnea and a systolic murmur. You suspect acute rheumatic fever. Which is a major criterion? p48, 85

A. Conjunctivitis

B. Carditis

C. Fever for 5 days

D. Erythema nodosum

52. A girl has migratory arthritis after a documented GAS infection. What is true about this condition? p94 partial recall

A. Joints are not painful

B. It heals without deformity

C. It affects large and small joints

53. A girl has persistent oligoarticular JIA involving four joints (knee, elbow, wrist, ankle) and is strongly ANA positive. What is the best next step? p87, 92 partial recall

A. Slit lamp examination (uveitis screening)

B. Monitor with ESR / ANA

Renal

54. In children with chronic kidney disease, what is the most effective strategy to slow progression? p47

A. Sodium bicarbonate for metabolic acidosis

B. BP control with ACE inhibitors

C. High-protein diet

D. Frequent diuretics

55. End-stage renal disease: what is the cause of the anemia? p56 partial recall

A. Inadequate erythropoietin

Only the answer was recalled in the source.

56. A 12-year-old girl with ESRD has bruising and nosebleeds. Platelet count is normal, bleeding time is prolonged.

Diagnosis? p57

- A. Immune thrombocytopenic purpura
- B. DIC
- C. Uremic platelet dysfunction
- D. von Willebrand disease

57. A child with renal failure missed dialysis and looks sick.

What will his blood gas show? p74

- A. High anion gap metabolic acidosis
- B. Metabolic alkalosis
- C. Respiratory acidosis

58. A patient had AKI and then developed HUS. His labs will show? p95 partial recall

- A. Hypocalcemia

Only the answer was recalled in the source.

59. A child's BP is 114/74, between the 50th and 90th percentile. What is the next step? p74 partial recall

- A. Start diet and healthy lifestyle
- B. Start antihypertensive treatment
- C. Reassure the parents that it is normal

60. A patient has BP 120/70 (90th-95th percentile) and BMI 25. What is the next step? p95 partial recall

- A. Lifestyle modification and more physical activity

Only the answer was recalled. The earlier question (p15) with BP 90th-95th and no obesity keyed "repeat BP on 2 more occasions".

61. On urine dipstick, which of the following is heme-negative (red urine without heme)? p81 partial recall

- A. Alport syndrome
- B. Urate crystals
- C. Post-streptococcal glomerulonephritis

62. A 6-year-old girl comes to the ER with red urine. She is otherwise healthy with no symptoms. What could cause this dipstick reading? p94 partial recall

- A. Myoglobinuria
- B. Urate crystals

The dipstick result itself was not recalled in the source.

63. A 10-year-old has periorbital and pitting edema, frothy urine, hypertension, hypoalbuminemia and hypercholesterolemia. Most likely diagnosis? p79 partial recall

- A. Minimal change disease

The source says this question was deleted from the exam because many got it wrong. Hypertension is not typical of minimal change disease.

64. What is true about minimal change disease? p89, 92

- A. Most common nephrotic syndrome in childhood
- B. Normal lipids and cholesterol
- C. Resistant to steroids
- D. Commonly causes ESRD in 5-10 years

65. A patient has abdominal pain, joint pain and a rash on the lower limbs (Henoch-Schonlein purpura). What should you monitor? p72 partial recall

- A. IgA
- B. Urinalysis

66. Palpable purpura on the buttocks and lower limbs with joint and abdominal pain (HSP). What will labs or biopsy show? p79, 83, 85, 92 partial recall

- A. IgA (vasculitis with predominant IgA deposits)
- B. IgG
- C. IgM

GI, nutrition, fluids

67. A 2-year-old (12 kg) with viral gastroenteritis has HR 130, otherwise normal vitals, looks tired, cries with slight tears, moist mucosa. Best rehydration strategy? p45

- A. D10 with normal saline 120 mL over 30 min
- B. Free water 500 mL over 2 hours
- C. Commercial drink, low carbohydrate, no sodium, 750 mL over 4 hours
- D. Oral solution with 1:1 glucose/sodium, 600 mL over 6 hours

68. Home oral rehydration solution contains? p80

- A. Half spoon salt + 6 spoons sugar in 1 L water
- B. Half spoon salt + 6 spoons sugar in 1 L apple juice
- C. Half spoon sugar + 6 spoons salt in 1 L apple juice

69. A child had rotavirus gastroenteritis. After going home the mother gave cow's milk and the diarrhea came back. What is appropriate? p66, 97 partial recall

- A. Give lactose-free milk for a period of time
- B. Give diluted milk until symptoms resolve

70. A 5-year-old boy has GERD that failed conservative management. What is the next step? p73 partial recall

- A. Initiate medical therapy
- B. Gastric fundoplication (source wrote "gastric arthroplasty")
- C. Diet

71. A 3-week-old girl has large vomits after breastfeeding and an epigastric mass. She is vigorous and crying. What is the gold standard for diagnosis? p59 partial recall

- A. Ultrasound
- B. Fluoroscopy
- C. X-ray

72. Goat's milk feeding can cause? p95 partial recall

A. Folate deficiency

Only the answer was recalled in the source.

73. When should vitamin D supplements be started in an exclusively breastfed baby? p82, 83 partial recall

A. Within the first month of life

Only the answer was recalled in the source.

74. A 16-month-old, exclusively breastfed, has swelling of the arms (wrists). Which labs are expected? p89

A. Low Ca, high PO₄, normal ALP, high PTH

B. Low Ca, low PO₄, low ALP, low PTH

C. High Ca, low PO₄, low ALP, high PTH

D. Low Ca, low PO₄, high ALP, high PTH

75. A child with moderate dehydration weighs 8 kg, Na 155. What is the maintenance fluid? p83

A. 0.9% saline D5 at 32 mL/h

B. 0.9% saline D5 at 42 mL/h

C. 0.45% saline D5 at 32 mL/h

D. 0.45% saline D5 at 42 mL/h

76. A 6 kg infant with 10% dehydration. Calculate the maintenance and deficit (mL/day). p95 partial recall

A. 600 maintenance, 600 deficit

Only the answer was recalled in the source.

77. A 3-year-old has moderate respiratory distress for 3 days. ABG: pH 7.25, CO₂ 50, HCO₃ 28. Interpretation? p57 partial recall

A. Respiratory acidosis

Only the answer was recalled in the source.

Toxicology

78. A 5-year-old (20 kg) accidentally ingested paracetamol. If the serum level is above the treatment line on the Rumack-Matthew nomogram, what is next? p49

A. Discharge with follow-up

B. Gastric lavage

C. N-acetylcysteine (NAC)

D. Activated charcoal

79. All of the following are true about salicylate poisoning EXCEPT: p73

A. Charcoal is effective

B. It causes respiratory acidosis and metabolic alkalosis

C. Urine alkalization increases elimination

D. Hyperglycemia followed by hypoglycemia

80. In salicylate poisoning, hyperglycemia (early) then hypoglycemia (late) is described. Which is an indication to give IV glucose? p86, 91, 102

A. Vomiting

B. Convulsions

C. Diaphoresis

D. Altered mental status

81. Which poisoning would NOT cause hypoglycemia at the beginning? p80, 85 partial recall

A. Salicylate

B. Acetaminophen

Neurology

82. A child has slurred speech and left facial twitching at night, with no focal deficit. What will the EEG show? p67

A. Occipital spikes

B. Centrotemporal spikes

C. Spikes on hyperventilation

D. 3 Hz spike and wave

83. A 13-year-old girl has episodes of staring and becoming motionless, 3 times a week, lasting 2 minutes, affecting school. During episodes she feels abdominal (epigastric) discomfort. Neuro exam is normal. Most likely cause? p90, 93, 103

A. Childhood absence epilepsy

B. Juvenile myoclonic epilepsy

C. Focal epilepsy

84. A patient had 2 generalized tonic-clonic febrile seizures within 6 hours, each lasting 7 minutes. What is the probability of epilepsy? p78

A. 0%, it is provoked

B. 1%, like the general population

C. Higher risk, about 6%, because it is atypical

D. Higher risk, 38%

85. An infant with tuberous sclerosis has clusters of sudden symmetric contractions on waking (infantile spasms). EEG shows hypsarrhythmia. Initial treatment? p78 partial recall

A. Vigabatrin (first line)

B. Other antiepileptic drugs

The exam showed a skin-findings image, not reproduced.

86. A patient with angiofibromas and seizures. Which is consistent with tuberous sclerosis? p88

A. Hypopigmented macules

B. Hyperpigmented macule on the back

C. Family history of renal carcinoma

87. A 4-year-old has axillary freckles, 6 cafe-au-lait spots and some neurological features. Diagnosis? p77, 97 partial recall

- A. Tuberous sclerosis
- B. Neurofibromatosis type 1
- C. Sturge-Weber

p88 variant: repeated seizures + cafe-au-lait spots = epilepsy with NF1.

88. A preterm had perinatal damage to the cortex and motor pathway. Which is the diagnosis? p88 partial recall

- A. Spastic cerebral palsy
- B. Dyskinetic cerebral palsy
- C. Mixed cerebral palsy

89. A newborn has marked enlargement of head circumference with a bulging anterior fontanelle, otherwise healthy. Next step? p87

- A. CT scan, craniosynostosis
- B. Reassure, familial variant
- C. Congenital hydrocephalus: transcranial ultrasound
- D. Seizure: EEG

90. A 9-month-old has a bulging fontanelle, head size above the 98th percentile and hydrocephalus. What is done next? p100 partial recall

- A. Head CT
- B. Start diuretics and re-measure later
- C. Head X-ray

Hematology, oncology, immunology

91. A child has petechiae and ecchymoses, no family history, platelets 33,000. Best therapy? p62

- A. Observe
- B. Steroids
- C. Immunoglobulin
- D. Prophylactic antibiotics

92. A 7-year-old has pallor, fatigue and hepatosplenomegaly. Labs: low Hb and low platelets. Most likely diagnosis? p63

- A. Wilms tumor
- B. Hodgkin lymphoma
- C. B-cell lymphoblastic leukemia
- D. Medulloblastoma

93. A child hit his knee on a car door and it swelled with blood (hemarthrosis). You suspect hemophilia A. What helps the diagnosis? p64 partial recall

- A. Prolonged PTT
- B. Low platelets

94. What is the cause of DiGeorge syndrome? p58, 79, 100

- A. Gene mutation of humoral immunity
- B. Absent thymus with impaired T-cell (cellular) function
- C. Natural killer cell defect
- D. Complement defect

95. A 4-year-old boy has otitis media and recurrent pneumonia. His uncle has the same problem. What test? p86

- A. Quantitative immunoglobulins
- B. T-cell flow cytometry
- C. X-ray for thymus
- D. Diphtheria antibodies

96. A boy has recurrent infections (pneumonia), eczema and easy bleeding. Which finding confirms the diagnosis? p90, 93 partial recall

- A. Small platelets on peripheral smear

Only the answer was recalled (Wiskott-Aldrich).

Endocrine, metabolic, growth, development

97. A girl with type 1 diabetes has fever, diarrhea and vomiting for 3 days. She is well hydrated. Glucose 260 mg/dL, bicarbonate normal, urine ketones positive. After adjusting her insulin, what is next? p64, 99

- A. Admit immediately
- B. Measure urine ketones daily
- C. Measure blood glucose daily
- D. Increase hydration

Key says A in both copies. I am not certain this is right: with normal bicarbonate and good hydration, sick-day management at home is often acceptable. Confirm with your doctor.

98. One month of polydipsia, polyuria and weight loss. Which will NOT be seen? p89

- A. Metabolic alkalosis
- B. Hyperglycemia
- C. Kussmaul breathing
- D. Dehydration

99. Hyperammonemia and metabolic acidosis. Which is a urea cycle defect? p65 partial recall

- A. Ornithine transcarbamylase (OTC)

Only the answer was recalled in the source.

100. An 8-year-old has not grown in height for 2 years. What is the normal prepubertal height increase per year? p75

- A. 1-2 cm
- B. 2-4 cm
- C. 4-8 cm
- D. 8-12 cm

101. A boy with a growth chart showing short height and delayed bone age. Most likely? p91 partial recall

- A. Turner syndrome
- B. Growth hormone deficiency
- C. Skeletal dysplasia

The growth chart was not reproduced in the source.

102. A 4-month-old: what can he do? p95 partial recall

A. Reaches for objects

Only the answer was recalled in the source.

103. A 3-year-old: what is expected? p81, 83 partial recall

A. Draws a cross and a square

B. Stands on one foot for 10 seconds

C. Speech is understood by strangers

The answer was written in Arabic in the source ("he talks and people understand him").

104. A school-age boy always interrupts class, does not like to sit, and does not follow rules. Diagnosis and treatment?

p56, 74 partial recall

A. ADHD: behavioral therapy

B. Oppositional defiant disorder

C. Autism spectrum: speech and language therapy

Answer conflicts in the source file

Same question on both sides of page 45, but with different highlighted answers. Worth 5 minutes because you already studied one version.

Biliary atresia: gold standard / definitive test

p19, p93, p98: liver biopsy. p55: intraoperative cholangiography. Both answers appear in the file. Ask your doctor which one they expect.

Episodic hematuria after URIs

p7, p32, p70: IgA nephropathy. p56: key says post-strep GN. Hematuria during or within a few days of a URI fits IgA nephropathy. PSGN comes 1-3 weeks after infection. The p56 key looks wrong.

GAS pharyngitis: antibiotics prevent which complication?

p33, p71: rheumatic fever. p95: key says post-strep GN. Antibiotics prevent rheumatic fever, not PSGN. The p95 key looks wrong.

10-week-old, Hb 10, well

p7, p29, p47, p74: physiologic anemia / reassure. p94: key says iron deficiency.

The p94 key looks wrong. This is physiologic anemia.

18-month milestone

p24: key "copies line". p95: key "helps to undress", and the source adds that the doctor said this is the right answer.

Go with the doctor's answer.

Croup-like case (2-year-old, noisy breathing, retractions)

p25, p68: inhaled epinephrine + steroid. p63: key "observe for hypoxia and dehydration".

The p63 source says it was not sure of the question. The earlier answer is the standard one.

Anion gap: which values do you need?

p30, p46, p63: sodium, chloride, bicarbonate. p52: highlight is on "Na, K, Ca, Cl".

The p52 highlight is an error.

IVIG and later vaccines

p32, p56, p100: "unchanged immune response" (p100 says not sure). p95: "immunoglobulin last month could affect MMR". IVIG can reduce the response to live vaccines (MMR, varicella), so these are usually delayed after high-dose IVIG. Inactivated vaccines are not affected. Ask your doctor which answer they expect.